

Stress Urinary Incontinence (Female)

Patient Information · Leaking with coughing, laughing, lifting or exercise

WARWIKI

Stress urinary incontinence (SUI) is leaking urine when something presses on the bladder, such as coughing, sneezing, laughing, lifting or exercise. In women it happens when the support under the urethra gets weak, often after childbirth or menopause. It is very common, and there are many ways to treat it.

About This Condition

The **urethra** (the tube urine flows through) normally stays closed when you cough or strain. The pelvic floor muscles and nearby tissue hold it in place. When that support gets weak, a sudden push from the belly forces urine out.

The key sign is **leaking with activity**. Leaking with a sudden, strong urge is a different problem called overactive bladder. Having both is called **mixed** incontinence.

What Causes It

- **Pregnancy and vaginal birth**, a leading cause
- **Menopause** and getting older
- Long-term cough, constipation or heavy lifting
- Extra body weight, smoking, and earlier pelvic surgery

Learn the Terms

Stress incontinence

Leaking urine with pressure, such as coughing, lifting or exercise.

Urethra

The tube that carries urine out of the body.

Pelvic floor

The muscles that support the bladder and urethra.

Kegels

Squeezing exercises that make the pelvic floor stronger.

Mixed incontinence

Having both stress leaks and urge leaks.

Pessary

A small support placed in the vagina to reduce leaks.

Urethral bulking

An injection that helps the urethra close.

Mid-urethral sling

A narrow mesh strip placed under the urethra during surgery.

Do I need surgery? Not always. Many women improve with pelvic floor exercises and simple changes, and some use a support device. Surgery lasts longer for many women. It is your choice, and you do not have to try everything else first.

How It's Diagnosed

- Questions about when you leak, and an exam. You may be asked to cough with a full bladder.
- A **urine test** to rule out infection, and a check of how well you empty
- Sometimes bladder testing (urodynamics) before surgery

How It's Treated

- 1 **Pelvic floor exercises**, best with a physical therapist. Also weight loss if you are overweight, less caffeine if you also have urgency, and treating cough or constipation.
- 2 **Support devices**: a pessary or a store-bought insert.
- 3 **Urethral bulking**: a quick injection. It carries less risk, but it works less well than surgery and often needs repeating.
- 4 **Surgery**: a mid-urethral sling, a sling made from your own tissue, or a Burch stitch repair. In 2019 the FDA stopped the sale of mesh placed through the vagina to repair prolapse. That order did not cover mesh slings for leakage, which are still FDA-cleared.

Each option has its own handout.

Living With SUI

- Learn to **squeeze before you cough, sneeze or lift**.
- Do your exercises every day. Give them at least 3 months.
- Treat constipation and long-term cough, which add pressure.

Call the office that is treating you if you have:

- Burning when you urinate, or a temperature of **100.4 °F (38 °C)** or higher
- Blood in your urine
- A feeling that your bladder is not emptying

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

1. SUI is leaking with pressure because the support under the urethra is weak.
2. Exercises help many women, but they take months.
3. Surgery is a choice, not a must.